

Reviewing and Obtaining Informed Consent

4.1 Overview

All study participants (Cases and Controls) must give their informed consent to take part in the study. This is a requirement of the Office for Human Research Protection of the United States Department of Health and Human Services and the regulations published in the Code of Federal Regulations (45 CFR 46). Obtaining informed consent is also a requirement of the collaborating Ghanaian institutions. The consent process provides the mechanism by which an individual can make an informed decision regarding participation in the study and also provides for the protection of a participant's rights as a subject in human research.

This chapter describes the procedures for obtaining informed consent.

4.2 Consenting Process

During this process, the study staff explains the contents of the consent document and its purpose and answers any questions which the potential participant may have regarding the study and her involvement in the study. Staff should be thoroughly familiar with the consent form documents and the procedures for obtaining informed consent.

4.3 Obtaining Consent

There are essentially two versions of the consent form for the study – one for Cases and one for Controls. However, to accommodate the need for hospital-specific information, each institution has its own Case version and Control version of the consent form (See Appendices 4-1, 4-2, 4-3, 4-4, 4-5, and 4-6.). The Informed Consent forms are available in both English and Twi. The six unique versions of the consent forms are as follows:

- Komfo Anokye Teaching Hospital – Informed Consent Form (Cases);
- Korle Bu Teaching Hospital – Informed Consent Form (Cases);

- Peace and Love Hospital – Informed Consent Form (Cases);
- Komfo Anokye Teaching Hospital – Informed Consent Form (Controls);
- Korle Bu Teaching Hospital – Informed Consent Form (Controls); and
- Peace and Love Hospital – Informed Consent Form (Controls)

For the pilot feasibility phase, KBTH handled the recruitment of Controls for the Accra area, and KATH initially handled the majority of recruitment of Controls for the Kumasi area. PLH began Control recruitment toward the end of the pilot effort and at that time a Control consent form specific to PLH was created and approved by the required IRBs

The Interviewer reviews the consent form with each potential participant. The Interviewer explains the purpose of the study and reviews all components of the study with the potential participant. The potential participant should be instructed to read the consent form carefully. It might be suggested to her that she underline any words that are not recognized or understood, or that she stop and ask questions as the document is read. The potential participant should be given ample time to read the document. If she has trouble reading the document, the Interviewer should read the document aloud exactly as it is written. The potential participant should have the opportunity to ask questions about what participation in the study involves.

After reviewing the Informed Consent with the potential participant, the Interviewer must be certain that the potential participant understands:

- The purpose of the study,
- What constitutes participation in the study including the questionnaire and biospecimen collection procedures,
- The potential risks and benefits associated with the study,
- Her rights and responsibilities as a study participant,
- That she can refuse to participate in selected components, and
- The voluntary aspect of the study and the fact that they can withdraw from the study at any time.

After the individual has read the consent document, it is important to invite questions from the potential participant. The Interviewer should offer to explain any words or phrases that may be unclear.

Once it is established that the potential participant has a clear understanding of the study requirements, the Interviewer will ask the participant to provide their consent to each component of the study. The participant will be asked to initial each of the following components: Interview, Measurements, Saliva Sample, Blood Sample, Stool Sample, Data and Sample Sharing, and Future Research. In addition, potential Cases will be asked to consent to providing a tissue sample taken at biopsy or surgery and access to their medical information.

The potential participant will be asked to sign and date two copies of the Informed Consent document. The Interviewer and one witness will also sign and date both copies of the informed consent document. One copy of the consent form should be given to the participant for their records and the other copy should be filed in the participant's folder.

4.4 Avoiding Refusals

There are several reasons why women may be hesitant to cooperate. It is the responsibility of the Interviewer to attempt to identify why a person may be reluctant. The Interviewer can be influential in the participant's motivation and in the success of participation. If the Interviewer is interested in the survey and enthusiastic about the study, these feelings are usually communicated with positive effects.

Below are several objections women may raise and the accompanying strategies to gain maximum cooperation. It is always helpful to preface a rebuttal to a woman's objection with a token agreement or understanding of her viewpoint, for example, "I can understand that," or "you certainly have the right to feel that way." This takes the participant off the defensive and usually works equally as well as a more detailed rebuttal would.

- **Objections based on not knowing the purpose of the study.** People tend to fear things they do not understand. By knowing the purpose of the study and for what purposes the data will be used, the Interviewer can alleviate many fears. It is important to immediately identify yourself as an Interviewer with the Ghana Breast Health Study. Always have your ID badge visible.

- **Verifying the legitimacy of the study.** If the participant wants to verify the purpose of the visit, the Interviewer should arrange for the participant to speak to the Study Manager.
- **Personal questions and confidentiality fears.** Everyone working on the study will maintain the confidentiality of the women. The Interviewer should make sure that the potential participant understands that confidentiality will be maintained.
- **Hostility toward the sponsor.** If the participant indicates any hostility toward any of the hospitals involved in the study, the Interviewer may want to reiterate the purpose of the study and the use of the information. The Interviewer can also tell the participant that her views and experience may get overlooked if she decides not to participate in the study.
- **Costs in time and energy.** The timing of the visit may also affect the participant's level of cooperation. If the potential participant is adamant that the timing is not good, the Interviewer should try to find a way to have the woman come back to the hospital or clinic on another day.

In summary, just as one participant differs from another, the reasons for refusals are many and varied. Some participants will not refuse outright but may express some hesitancy, reservations, or even initial hostility. In a short time, the Interviewer will become sensitive to the firmness of a respondent's "no" by the tone and wording of her comments. When the Interviewer begins to sense reasons behind the hesitancy, the Interviewer can develop ways to reassure the participant. Avoiding Refusals and Refusal Conversion is also discussed in Sections 5.2 and 5.3, respectively, in the Interviewer Manual.

4.5 Handling Refusals

The validity of all studies depends in large measure on very high participation rates. The Interviewer must be prepared to make every effort to turn a refusal into participation. Whenever possible, the Interviewer should try to find out why the participant is hesitant to participate and attempt to encourage participation. The Interviewer should attempt refusal conversion, as discussed in Section 4.4, but must be sensitive to the participant's wishes and not press her.

If the Interviewer has attempted refusal conversion and the potential participant still refuses participation, the Interviewer must document the consent refusal and the reason for refusal on the Recruitment Outcome Form (Appendix 4-7). Completion of the Recruitment Outcome Form under this circumstance is referenced in Section 4.6.1 of the Interviewer Manual.

4.6 Formal Refusal Conversion Efforts

For the pilot feasibility phase, after Control recruitment was completed a formal effort at refusal conversion was undertaken at KBTH and KATH to increase the participation rate. Interviewers returned to the field with the following instructions to attempt to encourage participation of Controls who had refused.

4.6.1 Protocol for Refusal Conversion

Staff should approach Controls who were located but refused to participate due to lack of time, unavailability at the time of initial approach, or lack of interest. Try to coordinate a convenient time to have them come to the clinic to complete the questionnaire, anthropometry, and biological specimen collection. If a clinic visit cannot be arranged, try to collect questionnaire data, biologic samples (at a minimum saliva) and waist and hip circumference at an appropriate location in the field. As a last resort, obtain critical questions on the questionnaire.

- **Which Controls to Approach**

With approval from NCI, Westat generated a list of selected Controls to be approached for refusal conversion. Westat included potential Controls who did not have time, were away from the residence at the time of approach, had another reason for not wanting to participate at the time they were previously approached, or were not interested.

Controls who could not be located, were too ill to participate, or were located but found to be ineligible (i.e. male, deceased, does not live in catchment area, out of eligible age range) were excluded from selection for refusal conversion

Priorities were assigned based on the highest likelihood of successful conversion; the greatest effort should be placed on the highest priority groups. In determining the logistics for the order of approach, however, the sites were asked to identify a plan that makes the best use of time and resources (for example, approaching controls in close proximity to one another on the same day, regardless of priority group).

Westat sent to the Study Managers a Refusal Conversion Log spreadsheet listing the PIDs selected for refusal conversion, the date of their initial approach (Date PID Assigned), district of residence, reason for refusal, and priority category. Study Managers were asked to use the log to record notes about refusal conversion attempt status, the outcome, and the date of conversion.

▪ **Method of Approach**

An interviewer should go out to locate the subject, re-introduce the study, and request participation.

- Make up to 5 contact attempts, preferably in person. This means the interviewer should make up to 5 attempts until talking with the woman. Once she has been located, please only talk with the woman once to try to convince her to participate.
- Send a different interviewer than the interviewer who made the initial contact that resulted in refusal. If a male interviewer or census worker made the initial contact, try to send a female interviewer.
- Explain that we understand they were approached previously about the study, but we were hoping that now they might have time available to participate. Explain that their participation is very important to help us understand more about breast health in Ghana and that their time would be very valuable and greatly appreciated.
- If the subject is willing to proceed, administer the Eligibility Screener.

▪ **Forms and Supplies to Bring**

- Study brochures
- The original Recruitment Outcome Form with the PID already assigned. ***Do not assign new PIDs.***
- A consent form
- Contact Information Form (Version 2)
- Eligibility Screener Form (Version 2)
- Risk Factor Questionnaire (Version 2)
- Specimen Kit
- The sheet of PID labels for the subject
- A canvas tote bag and stool collection (cosmetic) bag
- Tape measure
- Anthropometry Form

▪ **Completion of Study Components**

Preferred Approach: Schedule a Study Visit at the Hospital. The highest preference would be to arrange for the Control to come to the hospital to complete the study interview and specimen collection. Offer to assist with transportation and flexibility in scheduling. Indicate that she will be reimbursed for her travel and that while at the hospital a snack will be provided. The protocol for completing the study visit in the hospital is the same as the protocol outlined throughout this Manual.

Secondary Approach: Complete Study in the Field. If the Control does not have time to come to the hospital, offer to complete the study in the field. A few modifications for completion in the field will be necessary:

- **Conducting interview in the field.** Offer to conduct the interview in a private setting near her home such as a community center. It is necessary that the interview be confidential and private; her home may be used if there is a space where the interview can be conducted in private. Try to complete the entire interview. If the subject does not have time for the complete interview, as a last resort ask only critical items.
- **Collecting specimens in the field.** At a minimum, try to collect a saliva sample. If a phlebotomist is available to accompany the Interviewer on refusal conversion attempts in the field, a blood sample may also be obtained. Do not collect a stool sample in the field.
- **Collecting anthropometry in the field.** Use the tape measure to measure waist and hip circumference according to protocol and record the measurements on the Anthropometry Form. Weight, standing height, or sitting height cannot be measured in the field as the equipment will need to remain at the hospital for use by Cases. The exception to this is for PLH which has a second set of anthropometry equipment for use in the field.

■ **Tracking, Documentation and Data Entry**

- Use the Record of Calls to document contact attempts and outcome.
- Update the Refusal Conversion Log to track the status and outcomes for NCI. Record details/notes, whether the subject agreed to participate, and if so, the date of conversion.
- Update the Recruitment Tracking Log with changes to eligibility status and update if the questionnaire, anthropometry, and specimens were collected.
- Update the Recruitment Outcome Form to indicate any change in outcome. Clearly cross out old information, circle new information, and initial and date any changes.
- Be sure to place a PID on all of the study forms and make sure all match the PID label on the Recruitment Outcome Form.
- If any specimen is collected and therefore a master specimen kit is assigned, retain all three specimen forms (saliva, blood, and stool) even if some of the specimens are not obtained.
- The Study Manager will communicate with Westat when the refusal conversion is completed to request re-opening of data entry and to arrange for re-scanning of forms.